

Standard of Care for Tele-Mental Health Services

Document Title:	Standard of Care for Tele-Mental Health Services		
Document Ref. Number:	DoH/ST/SCTMHS/HQS/V1/2026	Version:	V1
New / Revised:	New		
Publication Date:	June 2026		
Effective Date:	June 2026		
Document Control:	DoH Strategy Sector		
Applies To:	DoH Healthcare Providers that are licensed to provide mental health services.		
Owner:	Healthcare Quality Sector		
Revision Date:	June 2029		
Revision Period:	Three years		
Contact:	hcps@doh.gov.ae		

1. Standard Scope and Purpose

- 1.1. This document specifies the service requirements for the DoH Standard of Care for Tele-mental health services.
- 1.2. The purpose of this document is to define the regulatory requirements for the safe, ethical and high-quality provision of mental health services delivered remotely using telecommunications technology. It is not to replicate the office but rather to enhance services through accessibility.
- 1.3. This document aims to:
 - 1.3.1. Improve access to mental health services across geographic and service barriers
 - 1.3.2. Ensure equivalence of quality between tele-mental health and in-person mental health care
 - 1.3.3. Safeguard patient confidentiality, data security, and privacy
 - 1.3.4. Establish clear clinical governance, documentation, and operational requirements
 - 1.3.5. Define provider qualifications, scope of practice, and accountability
- 1.4. These service requirements apply to:
 - 1.4.1. Healthcare providers licensed by DoH in the Emirate of Abu Dhabi delivering tele-mental health services
 - 1.4.2. All psychiatrists, psychologists, and mental health professionals providing psychiatric consultation remotely
 - 1.4.3. All digital platforms and technology systems used to deliver tele-mental health services, which must comply with Federal Law No. 2 of 2019 on the use of Information and Communication Technology (ICT) in Health Fields and DoH data security standards.
 - 1.4.4. All patients receiving tele-mental health services
- 1.5. This standard must be read in conjunction with the cabinet resolution no. 40 of 2019 concerning the implementing regulations of federal decree law no. 4 of 2016 on medical liability and its annexes, federal law no. 10 of 2023 concerning mental health, the DoH Standard for Tele-medicine, Standard for Specialized Mental Healthcare Services, Standard of Care for Depression and Anxiety Disorders and Standard of Care for ADHD.

2. Definitions and Abbreviations

No.	Term / Abbreviation	Definition
2.1	ADHICS	Abu Dhabi Information and Cybersecurity Standard
2.2	AAMEN	Abu Dhabi Healthcare Information Security Program to elevate cybersecurity across the healthcare ecosystem
2.3	DoH	Department of Health; The regulatory body of the Healthcare Sector in the Emirate of Abu Dhabi, Established based on law No. (10) of 2018
2.4	EHR	Electronic Health Record; a systemized digital collection of patient health information
2.5	ICT	Information and Communication Technology: refers to all technical means used to handle information and aid communication, encompassing computer hardware, software, networks and wireless devices
2.6	Malaffi	Healthcare Information Exchange platform, securely connecting all healthcare providers in Abu Dhabi
2.7	PQR	Professional Qualification Requirement: Requirements that provide the base for the authorities to assess credentials and relative documents submitted by applicants, in accordance to the UAE federal laws and benchmarked with international best practices
2.8	Tele-prescribing	Prescribing of medication by physician to patient in a different geographic location using Information and Communication Technology
2.9	Tele-mental health	Mental health remote-based patient-centered service that can include consultation, counseling, prescription and health monitoring by a psychiatrist or psychologist enabled by a range of telecommunications media such as phone, internet-based video or other electronic-based communications

3. Standard Requirements and Specifications

3.1 Licensure Requirements

- 3.1.1 A DoH license is required for the provision of tele-mental health services.
- 3.1.2 A facility may seek a license to provide tele-mental health through:
 - 3.1.2.1 Application for a new facility license to provide tele-mental health services standalone.
 - 3.1.2.2 Application to add tele-mental health service under an existing DoH licensed facility.
- 3.1.3 Facilities providing tele-mental health services need to fulfill one of the following facility types, with tele-mental health explicitly listed as a service:
 - 3.1.3.1 'Tele-Medicine Provider' facility type + Tele-mental health service name or facility sub-type
 - 3.1.3.2 'Center' facility type + Tele-mental health service name
 - 3.1.3.3 'Hospital' facility type + Tele-mental health service name

- 3.1.3.4 'Clinic-Specialized' facility type + Tele-mental health service name.
- 3.1.4 The provision of tele-mental health services requires the following minimum specialties:
- 3.1.4.1 Psychiatry (DoH licensed) or,
 - 3.1.4.2 Psychology (DoH licensed)
 - 3.1.4.3 Additional counseling-based allied health specialties can be included such as social work
- 3.1.5 The healthcare facility shall be fully integrated with the Malaffi health information exchange platform, ensuring real-time transmission of medical record data.
- 3.1.6 The healthcare facility shall comply with all the regulatory requirements including, but not limited to, healthcare provider manual, healthcare professional manual, complaint management, consent, laboratory, Infection control, first aid training, waste management, fire safety, Unified Healthcare Professional Qualification Requirements (PQR), Medication management, Abu Dhabi occupational safety and health system framework, Abu Dhabi Healthcare Information and Cyber Security [ADHICS] Standard, DoH Data and Digital Governance Standards and DoH standard on Telemedicine.
- 3.1.7 The healthcare facility shall comply with the requirements of Abu Dhabi Healthcare Information Security Program and shall be AAMEN Certified.
- 3.1.8 All diagnosis, counseling and treatment elements of tele-mental health services must comply with the specifications outlined in Cabinet Resolution No. 40, 2019 Governing Tele-Health Services.

3.2 Staff Qualifications & Training Requirements

- 3.2.1 All healthcare professionals must hold an active DoH professional license and work within their scope of practice and as per their granted privileges.
- 3.2.2 All relevant DoH-licensed healthcare professionals must complete the Information and Cyber Security Awareness courses assigned through the Abu Dhabi Healthcare Cyberlearning Program
- 3.2.3 All healthcare professionals involved in tele-mental health services must complete and maintain up-to-date training in quality improvement and patient safety, including methodologies such as incident reporting, root cause analysis, risk mitigation strategies and safe telehealth practices as an integral part of their ongoing professional development.
- 3.2.4 All healthcare providers of tele-mental health services must demonstrate competency in:
- 3.2.4.1 Telehealth ethics
 - 3.2.4.2 Digital confidentiality
 - 3.2.4.3 Crisis management in remote settings
- 3.2.5 Healthcare providers of tele-mental health must ensure that it:
- 3.2.5.1 Assesses its staff's training needs and provides training in telehealth technologies, where identified as necessary.
 - 3.2.5.2 Maintains up to date records of its staff's telehealth training, including the competencies mentioned in 3.2.4, whenever conducted.

3.3 Scope of Services

Tele-mental health may be used for

3.3.1 Initial Mental Health Assessment; permitted when:

- 3.3.1.1 Adequate clinical evaluation can be conducted remotely
- 3.3.1.2 Patient identity can be verified
- 3.3.1.3 Risk assessment is possible

3.3.2 Follow Up Consultations; including:

- 3.3.2.1 Medication monitoring
- 3.3.2.2 Psychotherapy and psychoeducation
- 3.3.2.3 Progress review

3.3.3 Psychotherapy; including:

- 3.3.3.1 Cognitive Behavioral Therapy (CBT)
- 3.3.3.2 Psychodynamic therapy
- 3.3.3.3 Supportive therapy
- 3.3.3.4 Behavioral therapy

3.4 Services Not Appropriate for Tele-mental health:

Tele-mental health should not be used when:

- 3.4.1 Immediate physical examination is required,
- 3.4.2 Severe cognitive impairment prevents meaningful participation,
- 3.4.3 There is significant risk of violence or self-harm without emergency support,
- 3.4.4 Referral to in-person services is required if any of the above criteria is met,
 - 3.4.4.1 The patient must be educated on the reasons the service is inappropriate for him/her.

3.5 Services Requiring Special Considerations for Tele-mental health:

Patients with the criteria below must meet the requirements outlined below:

3.5.1 Patients experiencing acute psychiatric emergencies

- 3.5.1.1 Tele-mental health can only be offered for psychiatric emergencies if the provider has an affiliated physical facility offering specialized mental health care.
- 3.5.1.2 Any such cases should be promptly handled as per escalation procedure for immediate transfer if needed.

3.5.2 Severe cognitive or sensory impairment

- 3.5.2.1 Patients with severe cognitive impairment preventing meaningful participation can receive tele-mental health services after an initial assessment is completed in-person.
- 3.5.2.2 All consultations must include the patient's primary caretaker or next of kin while the patient is observed via video.

3.6 Technological Requirements

- 3.6.1 DoH licensed tele-mental health service providers must also have the right physical infrastructure for the tele-mental health areas to preserve the privacy, access to and confidentiality of case details
- 3.6.2 Providers must comply with Information and Communication Technology (ICT) requirements outlined in the DoH Standard on Tele-Medicine and Cabinet Resolution No. 40, 2019 Governing Tele-Health Services including:
 - 3.6.2.1 Providers of tele-medicine services must have in place data management systems and protocols that maintain the confidentiality of gathered personal/patient information and that access to, use, sharing and transmission and reporting of such data complies with relevant DoH regulations.
 - 3.6.2.2 Ensure the availability of the right ICT infrastructure and supportive environment for the tele-medicine service provided including:
 - 3.6.2.2.1 Availability of the requisite range of equipment and that equipment choice, maintenance and servicing support quality and timely or real-time, as appropriate, transmission of relevant information and/or data and/or medical imaging results.
 - 3.6.2.2.2 ICT Policies, procedures and systems to assure the safe, effective and secure transmission of health information and its confidentiality.
 - 3.6.2.2.3 Regular monitoring and assessment of the ICT infrastructure and supportive environment to assure their effectiveness.
 - 3.6.2.2.4 ICT policies, procedures and systems that meet the Abu Dhabi Digital Authority (ADDA) and DoH technical and security regulatory requirements and that afford a high degree of interoperability.
 - 3.6.2.2.5 Protocols for network security, confidentiality, privacy and connectivity and protection of confidentiality with regular assessments to ensure the effectiveness of all systems.
 - 3.6.2.2.6 Systems in place to ensure sufficient availability of the network for critical connectivity.
 - 3.6.2.2.7 Standard procedures for any emergency event or technical failures for communication equipment, devices and IT systems to assure the security and protection of the patient health records.

3.7 Patient Identification and Verification

- 3.7.1 Prior to initiating any tele-consultation the healthcare professional must verify the patient's:
 - 3.7.1.1 Full name
 - 3.7.1.2 Date of birth
 - 3.7.1.3 Emirates ID
 - 3.7.1.4 Physical location
 - 3.7.1.5 Contact number
 - 3.7.1.6 Emergency contact

3.8 Informed Consent

- 3.8.1 The patient must provide explicit informed consent prior to initiating consultation
- 3.8.2 All consent must be documented in the patient's medical record and comply with DoH 'Patient Consent Standard'
- 3.8.3 All patients must be provided with a documented technical failure plan that clearly specifies the steps required in cases of technical difficulties
- 3.8.4 Informed consent must be renewed periodically every 3 months in addition to the following circumstances:
 - 3.8.4.1 Any changes to treatment
 - 3.8.4.2 Any changes to technology modality (e.g. video to audio)
 - 3.8.4.3 Any new risks that arise
- 3.8.5 Documentation of consent must include, but not limited to:
 - 3.8.5.1 Date and time of consent
 - 3.8.5.2 Name of provider obtaining consent
 - 3.8.5.3 Nature of tele-mental health
 - 3.8.5.4 Risks and limitations
 - 3.8.5.5 Confidentiality
 - 3.8.5.6 Emergency procedures
 - 3.8.5.7 Patient understanding of the information provided
 - 3.8.5.8 Patient's capacity for decision-making
 - 3.8.5.9 Patient agreement

3.9 Patient Confidentiality and Privacy

- 3.9.1 Healthcare professionals must ensure that tele-mental health consultations occur in private, secure environments.
- 3.9.2 Healthcare professionals must ensure no unauthorized individuals during tele-mental health consultations.
- 3.9.3 Healthcare professionals should encourage patients to participate from a private location and avoid public spaces.
- 3.9.4 Patients should make the necessary arrangements to ensure the privacy of their location.
- 3.9.5 Every patient must be informed of all his or her rights and responsibilities, including his or her right to protect the confidentiality of his or her information.
- 3.9.6 Recording of tele-mental health consultations is prohibited.
 - 3.9.6.1 In cases of recording for teaching purposes, patient consent must be obtained and clearly documented.

3.10 Clinical Standards

- 3.10.1 The clinical standard of tele-mental health must be equivalent to in-person care.
- 3.10.2 Special attention must be given to non-verbal cues including eye movement patterns, changes in posture, etc.
- 3.10.3 All teleconsultations require video + audio assessment. Cases of audio-only consultations require clear, documented clinical justification.

3.10.4 Assessment

Patient assessment must include, but not limited to

- 3.10.4.1 Patient suitability for tele-mental health services including, but not limited to:
 - 3.10.4.1.1 Patient comfort with remote care
 - 3.10.4.1.2 Privacy at patient location
 - 3.10.4.1.3 Ability to engage effectively
- 3.10.4.2 Presenting complaint including duration of symptoms and functional impact
- 3.10.4.3 Psychiatric history
- 3.10.4.4 Family psychiatric history
- 3.10.4.5 Medical history
- 3.10.4.6 Response to treatment (for follow-ups)
- 3.10.4.7 Medication side effects, where applicable
- 3.10.4.8 Medication history
- 3.10.4.9 Psychosocial assessment
- 3.10.4.10 Mental status examination including cognition, insight and judgement
- 3.10.4.11 Clinical interview
- 3.10.4.12 Physical symptoms; if physical examination is required, a referral for in-person assessment must be arranged
- 3.10.4.13 Risk assessment
 - 3.10.4.13.1 Suicide and safety risk must be conducted on initial assessment and repeated as clinically indicated.
 - 3.10.4.13.2 Assessment of violence risk and safeguarding concerns should be documented as clinically indicated.

3.10.5 Diagnosis

- 3.10.5.1 Healthcare professionals must use recognized classification systems such as Diagnostic Statistic Manual (latest version) or International Classification of Diseases.
- 3.10.5.2 The use of any standardized tools for assessment/ diagnosis must be clearly documented.

3.10.6 Treatment

3.10.6.1 The psychiatrist or psychologist must develop a **documented**, patient-centered treatment plan in collaboration with the patient that considers the patient's severity, complexity and preference for treatment modalities including a minimum of:

- 3.10.6.1.1 Patient's goals of treatment
- 3.10.6.1.2 Outcome measures
- 3.10.6.1.3 Baseline assessment or initial scores of any tools used e.g. pHQ-9, GAD-7
- 3.10.6.1.4 Treatment modalities to be used and the discipline responsible for each e.g. psychiatry, psychology
- 3.10.6.1.5 Planned review dates of treatment plan
- 3.10.6.1.6 Discharge framework
- 3.10.6.1.7 Referrals required to other multidisciplinary teams

3.10.6.2 When medication is prescribed, the following additional parameters must be included in the documented treatment plan

- 3.10.6.2.1 Review of the risks and benefits of the prescribed medication
- 3.10.6.2.2 History of previous treatment
- 3.10.6.2.3 Pharmacovigilance
- 3.10.6.2.4 Timeframe for therapeutic response
- 3.10.6.2.5 Patient consent, and his or her right to withdrawal
- 3.10.6.2.6 Discussion of the medication management plan with the patient providing information about reasons for offering pharmaceutical treatment, suitable choice and dose of medication (preference should be given to the use of generic medications), expectations upon initiating therapy, expected onset of therapeutic effects and expected or potential adverse or withdrawal effects

3.10.7 Documentation

3.10.7.1 All tele-mental health consultations must be documented in the Electronic Health Record (EHR) and must include, at minimum:

- 3.10.7.1.1 Date and time of consultation
- 3.10.7.1.2 Modality used (video/ audio). For non-video consultations must include documented justification
- 3.10.7.1.3 Identity verification confirmation
- 3.10.7.1.4 Informed consent confirmation
- 3.10.7.1.5 Clinical assessment as per 3.10.4
- 3.10.7.1.6 Diagnosis
- 3.10.7.1.7 Treatment plan as per 3.10.6.1 and 3.10.6.2

3.10.7.1.8 Prescriptions

3.10.7.1.9 Follow-up plans

3.10.7.1.10 All patient and family education including the topics covered, materials shared, and evidence of patient and/or family understanding.

3.10.8 Prescribing

3.10.8.1 To provide any prescriptions, tele-mental health services must have 'tele-prescribing' listed as a service type in the licensing of the provider.

3.10.8.2 Psychiatric medications prescribed through tele-mental health must comply with 'DoH standard for the Management of Narcotics, Psychotropic and Semi-Controlled Medicinal Products' and 'Standard for Prescribing High-Risk Controlled Analgesics'.

3.10.8.3 A sufficient, documented clinical assessment including comprehensive medication reconciliation must be conducted prior to prescribing any medication.

3.10.8.4 A clear, well documented treatment plan must be in place prior to prescribing any medication

3.10.8.5 All medications prescribed must be supported by the patient's diagnostic.

3.10.8.6 The following medications cannot be prescribed through tele-mental health:

3.10.8.6.1 Clozapine

3.10.8.6.2 Long-acting injectables

3.10.8.6.3 Ketamine therapy

3.10.8.6.4 Lithium

3.10.8.7 The following medications can only be prescribed via tele-mental health as a continuation of treatment following an in-person assessment for initiation:

3.10.8.7.1 Stimulants

3.10.8.7.2 Sedative-hypnotics (excluding hypnotic benzodiazepines)

3.10.8.7.3 Benzodiazepines

3.10.8.8 Stimulant safeguarding

3.10.8.8.1 Continuation of stimulant therapy via tele-mental health requires the following:

3.10.8.8.1.1 Evaluation of treatment response

3.10.8.8.1.2 Assessment of adverse effects

3.10.8.8.1.3 Screening for substance use disorder

3.10.8.9 Sedative-Hypnotics safeguarding

3.10.8.9.1 Continuation of Sedative-Hypnotic therapy via tele-mental health requires the following:

3.10.8.9.1.1 Documentation of sleep disorder

3.10.8.9.1.2 Monitoring of medication effectiveness and side effects

3.10.8.9.1.3 Documentation of comorbid psychiatric conditions

- 3.10.8.9.1.4 Documented justification of use beyond 4-week duration, in addition to
 - 3.10.8.9.1.4.1 Documented periodic reassessment of benefit vs. risk
 - 3.10.8.9.1.4.2 Documented shared decision-making, reflecting multidisciplinary care.
 - 3.10.8.9.1.4.3 Monitoring for dependence and tolerance

3.10.8.10 Benzodiazepine safeguarding:

- 3.10.8.10.1 Continuation of benzodiazepine treatment may be permitted if:
 - 3.10.8.10.1.1 The patient is stabilized on treatment
 - 3.10.8.10.1.2 History of medication misuse has been assessed including substance use disorder history and drug-seeking behavior
 - 3.10.8.10.1.3 National platform for controlled medication has been reviewed

3.11 Clinical Governance

- 3.11.1.1 All tele-mental health providers must comply with DoH tele-medicine requirements outlined in 'DoH Standard on Tele-medicine'.
- 3.11.1.2 Facilities providing tele-mental health must develop protocols and pathways that address:
 - 3.11.1.2.1 Patient eligibility for tele-mental health
 - 3.11.1.2.2 Prescribing practices and polypharmacy monitoring
 - 3.11.1.2.3 Suicide risk management
 - 3.11.1.2.4 Emergency escalation procedures
 - 3.11.1.2.5 Safeguarding procedures
 - 3.11.1.2.6 Confidentiality and information governance
 - 3.11.1.2.7 Incident reporting, including but not limited to:
 - 3.11.1.2.7.1 Adverse medication events
 - 3.11.1.2.7.2 Breaches of confidentiality
 - 3.11.1.2.7.3 Technology failures affecting patient care
 - 3.11.1.2.7.4 Safeguarding concerns
- 3.11.1.3 Facilities must develop systems to monitor the quality and safety of tele-mental health services, including but not limited to:
 - 3.11.1.3.1 Patient outcomes
 - 3.11.1.3.2 Documentation completeness
 - 3.11.1.3.3 Suicide risk assessment rates
 - 3.11.1.3.4 Follow up compliance
 - 3.11.1.3.5 Rates of non-video consultations
 - 3.11.1.3.6 Referral rates for in-person care
 - 3.11.1.3.7 Patient experience

3.12 Emergency & Crisis Protocols

- 3.12.1 Providers must have crisis intervention protocols with clear referral pathways for escalating care and coordination with emergency departments.
- 3.12.2 Telemedicine providers providing tele-mental health must have affiliations with providers within the ecosystem to facilitate in-person escalation of care.
- 3.12.3 Healthcare professionals must ensure the patient's emergency contacts are documented in the patient's Health Electronic Record.
- 3.12.4 Healthcare professionals must verify the patient's location at each consultation.

3.13 Additional Considerations for Children & Adolescents

- 3.13.1 Tele-mental health services for children and adolescents must be provided by a Child and Adolescent psychiatrist and/or child psychologist.
- 3.13.2 All tele-mental health consultations require video + audio assessment. Cases of audio-only consultations require clear, documented clinical justification.
- 3.13.3 Patients under the age of 18 years old require (documented) informed consent from a parent or legal guardian.
- 3.13.4 Children under the age of 14 years old require the parent or legal guardian to be present during the start of the consultation.
- 3.13.5 Children under the age of 10 years old require the parent or legal guardian to be present during the entire consultation.
 - 3.13.5.1 The clinician may request private discussion time with the minor, where developmentally appropriate.
- 3.13.6 Providers must conduct a safeguarding assessment for patients under the age of 18 years old which includes an assessment of:
 - 3.13.6.1 Abuse
 - 3.13.6.2 Neglect
 - 3.13.6.3 Bullying
 - 3.13.6.4 Exploitation
- 3.13.7 Patients not suitable for tele-mental health include:
 - 3.13.7.1 Very young children (<7 years old)
 - 3.13.7.2 Severe developmental disorders requiring physical observation
 - 3.13.7.3 Acute behavioral crises
- 3.13.8 Mandatory documentation for patients under the age of 18 includes, but not limited to:
 - 3.13.8.1 Minor's full name and date of birth
 - 3.13.8.2 Parent or guardian's name and relationship to the patient
 - 3.13.8.3 Parent or guardian's identity verification using minimum 2 identifiers e.g. (Emirates ID, passport number, date of birth)

- 3.13.8.4 Parent or guardian's contact details
- 3.13.8.5 Confirmation that tele-mental health was explained
- 3.13.8.6 Date and time of consent
- 3.13.8.7 Whether parent or guardian was present + duration of presence
- 3.13.8.8 Any private discussions with the patient excluding the parent or guardian
- 3.13.8.9 Explanation of confidentiality
- 3.13.8.10 Safeguarding assessment including any concerns identified and actions taken

4.Key stakeholder Roles and Responsibilities

- 4.1 Healthcare facilities shall meet the service specifications and requirements set out in this standard and other relevant federal and local regulations.
- 4.2 Healthcare professionals must provide services within licensed facilities that are equipped with the necessary resources, equipment, and support to ensure patient safety and quality of care, as per DoH licensing and accreditation requirements.
- 4.3 Billing and reimbursement shall be in accordance with Standard Provider Contract, DoH Mandatory Tariff and associated Claims and Adjudication Rules, and the Claims and Adjudication Standard. All documents are available from the DoH website: <https://www.doh.gov.ae/shafafiya>.

5.Monitoring and Evaluation

- 5.1 A monitoring and evaluation framework is in place to evaluate the effectiveness, outcomes, and impact of this Standard, and where necessary changes to ensure continuous improvement within the health system in line with emerging new developments in healthcare sciences, medical practices, and healthcare education and training.

Jawda KPIs for Outpatient Mental Health
Percentage of Patients on Antipsychotic Medication(s) Who Had Complete Routine Investigation and ECG performed within 12 months prior to prescription
Percentage of Patients Who Were Concurrently Prescribed Three or More Different Antipsychotic Medications
Percentage of Patients Who Were Concurrently Prescribed Two or More Different Benzodiazepines
Percentage of Patients Receiving Psychotherapy Who Achieved a Reduction in Depression Symptoms
Percentage of Patients Receiving Psychotherapy Who Achieved a Reduction in Anxiety Symptoms
Percentage of First Available Appointment for Patients Referred for Mental Health Services

6.Enforcement and Sanctions

- 6.1 DoH may impose sanctions in relation to any breach of requirements under this Standard in accordance with the disciplinary regulation of the healthcare sector.

7.Exempted from Scope

NA

8. Relevant Reference Documents

No.	Reference Date	Reference Name	Relation Explanation / Coding / Publication Links
1.	2020	DoH Standard for Tele-medicine	https://www.doh.gov.ae/en/resources/standards
2.	2019	Cabinet Resolution No. (40) of 2019	https://uaelegislation.gov.ae/en/legislations/1191/regulations/753/download
3.	2024	Standard for Specialized Mental Healthcare Services	https://www.doh.gov.ae/en/resources/standards
4.	2025	Standard of Care for Depression and Anxiety Disorders	https://www.doh.gov.ae/en/resources/standards
5.	2025	Standard of Care for ADHD	https://www.doh.gov.ae/en/resources/standards
6.	2021	Telepsychiatry Global Guidelines 2021	https://wpanet.org/wp-content/uploads/2025/07/FINAL_2021_WPA_Telepsychiatry-Guidelines-23Sep202111001.pdf
7.	2021	APA Position Statement on Telemedicine in Psychiatry	https://www.psychiatry.org/getattachment/a9405e26-3a69-4e9d-95b6-5df9568fb443/Position-Telemedicine-in-Psychiatry.pdf
8.	2023	Telehealth in Psychiatry	https://www.ranzcp.org/getmedia/2148dcf0-9fcd-4dc2-80bb-4bef2f44f2ca/ppg-19-telehealth-in-psychiatry-oct-2023-v2.pdf
9.	2024	Patient Consent Standard	https://www.doh.gov.ae/en/resources/standards
10.	2024	Abu Dhabi Healthcare Information and Cyber Security Standard – V2	https://www.doh.gov.ae/en/resources/standards
11.	2025	Standard for Prescribing High Risk Controlled Analgesics	https://www.doh.gov.ae/en/resources/standards

12.	2021	DoH Standard for the Management of Narcotics, Psychotropic and Semi-Controlled Medicinal Products	https://www.doh.gov.ae/en/resources/standards
-----	------	---	---